Request for Application:
Rural Health Transformation Hub Leadership Opportunity
For Rural Community Hubs for Chronic Care Innovation

I. Introduction

Introducing the Rural Community Hubs for Chronic Care Innovation Initiative

The Commonwealth of Kentucky has been awarded funding through the Centers for Medicare & Medicaid Services (CMS) Rural Health Transformation Fund (RHTF). This investment will empower Kentucky to launch and implement its Rural Health Transformation Plan (RHTP) — a community-driven strategy to expand access and improve health outcomes for rural residents across the Commonwealth. Read Governor Beshear's press release.

This project is 100% funded by the Centers for Medicare and Medicaid Services (CMS) of the U.S. Department of Health and Human Services (HHS) for a total of $212.9 million in budget year 1 with 100% by CMS/HHS. The RHTF grant spans five budget periods aligned with federal fiscal years (FY2026–FY2030). Funding is disbursed annually by CMS based on achievement of performance metrics. The contents are those of the author(s) and do not necessarily represent the official views of, nor an endorsement, by CMS/HHS, or the U.S. Government. View the Notice of Funding Opportunity from CMS.

You can find more information about KY's Rural Health Transformation Plan (RHTP) on the KY RHT website (and read the full application) here: https://ruralhealthplan.ky.gov/.

The Rural Community Hubs for Chronic Care Innovation (RCH) initiative is a component of Kentucky's RHTP, funded through a cooperative agreement awarded by CMS to the Kentucky Cabinet for Health and Family Services (CHFS).

The RCH initiative seeks to unite fragmented interventions, programs, resources, and technology tools for local community organizations to enable coordinated, community-led action across the chronic disease continuum – from early education and engagement to prevention, access to services, and ongoing chronic care management – with an initial focus on obesity and diabetes.

Foundation for a Healthy Kentucky's Role in the RCH Initiative

The Foundation for a Healthy Kentucky (FHKY) is a subrecipient of CHFS and will serve as a statewide, neutral convening and implementation partner for the RCH initiative. In this role, FHKY supports hub governance design, stakeholder engagement, facilitation, and administration of locally driven sub-awards, leveraging its long-standing relationships with rural communities and cross-sector organizations.

CHFS remains the prime recipient of the federal award and retains final authority over program requirements, approvals, and allowable uses of funds.

Purpose of this Request for Application

Through this Request for Application (RFA), FHKY is soliciting applications from eligible organizations to serve as a Rural Community Hub Lead Organization within one of the following Kentucky Area Development Districts (ADDs): Barren River, Kentucky River, Pennyrile, Cumberland Valley, Lincoln Trail, Bluegrass, Northern Kentucky, FIVCO, KIPDA, Green River, Buffalo Trace, and Gateway.

FHKY anticipates making up to one (1) award per ADD, for a total of up to twelve (12) awards under this RFA. Each applicant may submit only one (1) application and may apply to serve only one (1) ADD (refer to *Appendix M. Kentucky*

Area Development Districts (ADDs) in KY RHTP Other Supporting Documentation_CMS-RHT-26-001.pdf for a map of Kentucky's ADDs and the counties within).

For the first Federal Budget Period (BP1) (Rural Community Hub Launch; March 1, 2027 – September 30, 2027), anticipated funding may include up to $500,000 per ADD to support Rural Community Hub stand-up and operations ("Hub Operations funding").

Subject to Kentucky's successful award and continuation through all five federal budget periods, each Rural Community Hub may be eligible to receive up to $10 million in total funding across Federal Fiscal Years 2026-2030 *(refer to **V. Anticipated Schedule and Funding Performance-based continuation** below)*.

II. Rural Community Hub Lead Organization: Vision and Purpose

Key Definitions

- **Rural Community Hub (RCH):** The local "hub-and-spoke" collaborative network within an ADD, led by the Rural Community Hub Lead Organization and participating core partners and network participant organizations. The RCH serves as the primary working group of cross-sector organizations for driving collective efforts in each ADD, focused on obesity and diabetes prevention and management, integrating nutrition, physical activity programs, and digital self-management tools.
- **Rural Community Hub Lead Organization:** Leads the coordination of core partners and network participant organizations to drive collective efforts for each RCH. The RCH Lead Organization is responsible for establishing, convening, coordinating, and supporting RCH operations within the selected ADD, including serving as the pass-through entity for Initiative subawards.
- **Local Hub-and-spoke:** A descriptive term for the model in which the Lead Organization functions as the hub and participating partner organizations function as spokes at the ADD-level.
- **Core Partners:** Partner organizations across sectors that collaborate closely with the Hub by participating in decision-making, shaping strategy, sharing resources, and leading or implementing community Initiatives to advance shared goals.
- **Network Participant Organizations:** Partner organizations that get involved by engaging in program activities, offering services or insights, expanding community reach, and supporting targeted program components.

The RCH initiative brings together a broad network of organizations across Kentucky's rural health system to champion coordinated, community-driven action and continued systems transformation—sustained changes to stakeholder relationships, governance structures, policies, processes, and resource alignment that improve how communities prevent and manage chronic disease—with an initial focus on obesity and diabetes.

Rural Community Hub Lead Organizations will function as the central coordinating entities for each RCH—convening and aligning multisectoral organizations, facilitating agreement on shared strategy, and supporting local RCH program implementation.

Rural Community Hub Lead Organizations will bring together a group of core partners to decide on a set of strategic initiatives to implement within the ADD to address obesity and diabetes. These initiatives will have a separate funding stream that will be available at a later date. The RCH Lead Organization will facilitate governance and shared decision-making amongst core partners, foster partner alignment, convene stakeholders, and support the long-term sustainability of the RCH network and initiatives. In BP2, they will oversee the implementation of evidence-based chronic disease prevention and management initiatives, including project management, staffing, implementation support, and evaluation. Rural Community Hub Lead Organizations will help organize regional collaboration, facilitate joint planning

and learning, support continuous assessment for quality improvement, and coordinate shared resources and operations to advance community-driven health improvement efforts.

Rural Community Hub Lead Organizations will also support the thoughtful use of technology and data to strengthen coordination across chronic disease prevention and management efforts. Technology-enabled approaches will help organizations share information, improve care coordination, support learning, and better understand the impact of community-led initiatives.

This RFA is intended only for organizations who wish to apply to be the **RCH Lead Hub Organization**. Organizations that are not selected as a Rural Community Hub Lead Organization will have future opportunities to participate as a core partner, network participant organization, or subrecipient to implement one or more priority initiatives.

III. Scope of Work

Overview and Purpose

During BP1 (**Rural Community Hub Launch; March 1, 2027 – September 30, 2027**), the Rural Community Hub Lead Organization will be responsible for establishing, planning, launching, and supporting the initial operation of a RCH within the ADD. BP1 includes convening core partners; establishing RCH partnership governance, operations, and infrastructure; collaboratively designing and preparing evidence-informed obesity and diabetes initiatives; supporting early implementation activities; and serving as the pass-through entity for initiative-level subawards in compliance with federal requirements.

BP1 Objectives

Through BP1 activities, the Rural Community Hub Lead Organization is expected to:

- Establish and operationalize a collaborative RCH that convenes local, multi-sector core partners and network participant organizations to coordinate obesity and diabetes prevention and chronic disease management efforts.
- Convene core partners in BP1 to identify evidence-informed, community-led approaches to chronic disease prevention in BP2, with an initial focus on obesity and diabetes and an emphasis on upstream and primary prevention.
- Strengthen cross-sector collaboration and partnerships by aligning health care, public health, and community-based organizations at the ADD-level.
- Build sustainable local capacity through workforce development, technical assistance, and scalable operating models.
- Support interoperable data sharing and care coordination to improve referrals, track outcomes, and enable continuous learning.
- Leverage appropriate technology tools to support prevention, care coordination, and initiative implementation.

BP1 Activities

Rural Community Hub Lead Organizations must complete the following BP1 activities.

1. RCH Establishment and Organizational Ramp-up

- Establish baseline RCH governance, administrative, and operational structures (leveraging guidance and tools provided by FHKY and CHFS).

- Hire or assign staff necessary to support day-to-day RCH operations and coordination as identified in your budget.
- Employ an RCH Operations Lead and two community support workers by April 1, 2027.
- Conduct outreach activities across the ADD to raise awareness of the RCH initiative and recruit RCH core partners and network participant organizations.
- Onboard RHTP-related technology platforms, such as care coordination tools, in collaboration with FHKY and CHFS.
- Establish and document governance structures, including decision-making processes, partner roles and responsibilities, accountability mechanisms, contingency plans for material implementation challenges and risks, and procedures for shared ownership and oversight of RCH activities.

2. Partner Convening and Engagement

- Convene a minimum of ten (10) cross-sector core partners, representing all counties across the selected ADD, to participate in structured initiative planning activities.
- Recruit, onboard, and actively engage RCH partners on an ongoing basis; onboard a minimum of twenty (20) RCH network participant organizations during BP1.
- Formalize partner participation through governance, planning, and decision-making structures that clearly define partner responsibilities, contributions, and accountability for advancing shared RCH goals.
- Facilitate operational huddles and steering meetings for RCH core partners and initiative leadership, including documentation of agendas, decisions, action items, and progress.
- Complete annual Partner Survey and facilitate distribution of the Survey broadly amongst current and potential community partners.

3. Initiative Planning and Design

- In collaboration with core partners, FHKY, and CHFS staff, design two (2) to four (4) evidence-informed obesity and diabetes initiatives for deployment during the first year of RCH operations.
- Utilize community input, existing data sources (e.g., Community Health Assessments), and evidence-based practices to inform initiative prioritization and design.
- Support completion of environmental scans to inform initiative planning and design (as needed).

4. Initiative Implementation Planning and Budget Development

- For each proposed initiative, develop an implementation workplan and associated budget that demonstrates readiness to plan and launch the initiatives, using the ”Operational Blueprint” planning worksheet as provided by FHKY. At a minimum, the implementation plan for each proposed initiative must include:
    - Clear initiative objectives and intended outcomes, aligned with RCH goals and CMS requirements.
    - Key activities and quarterly milestones for BP1.
    - Staffing and partner roles and responsibilities.
    - Budget and budget narrative, in compliance with CMS budget guidance, the SF-424A (Budget Information for Non-Construction Programs), and the applicable Uniform Administrative Requirements, Cost Principles, and Audit Requirements under 2 CFR 200.
    - Approach to performance management, including proposed Objectives and Key Results (OKRs). Sustainability plan demonstrating how the initiative may be sustained or scaled over time.

5. Subrecipient Management and Pass-through Responsibilities

- Distribute initiative funding to subrecipients.
- Serve as the pass-through entity for all RCH initiative subawards in compliance with 2 CFR 200.332, including execution of subrecipient agreements.
- Conduct subrecipient risk assessment, monitoring, and technical assistance.
- Coordinate closely with FHKY and CHFS staff to ensure subrecipient compliance with federal and program requirements.

6. Reporting, Performance Management, and Oversight

- Maintain systems and processes to document programmatic and financial activity.
- Submit programmatic and financial reports to FHKY in accordance with CMS and subaward requirements.
- Maintain documentation for OKRs, including data sources, assumptions, and limitations.
- Monitor and document implementation risks, mitigation activities, governance decisions, and corrective actions; escalate significant risks to FHKY and CHFS as appropriate.
- Aggregate and synthesize performance information, both quantitative and qualitative, across RCH partners and initiatives to support reporting. Collect and share data to support reporting on program-wide Objectives and Key Results (OKRs) using frameworks and methodologies to be defined by FHKY. Program-wide OKRs include:
  - Rural Community Hub Coverage: # of Area Development Districts (ADDs) with a Rural Community Hub (RCH) (An RCH is considered active in an ADD upon award of an RCH Lead Organization for that district.)
  - Stakeholder Engagement: # of committed partners in each RCH (Committed partners have formally indicated their intent to engage in the RCH by completing and submitting the standardized intake/commitment form.)
  - Local Initiative Coverage: # of chronic disease or rural health initiatives launched locally via RCHs with multi-stakeholder engagement.
  - Lives Impacted through RCH-supported Initiatives: count of individuals impacted by an RCH enabled program or intervention.

BP1 Deliverables and Anticipated Timing

The table below summarizes key BP1 deliverables and anticipated timing associated with the activities described above. These deliverables correspond to BP1 (**Rural Community Hub Launch; March 1, 2027 – September 30, 2027)** and are intended to illustrate anticipated outputs and sequencing.

Activity Category	Key Deliverables	Anticipated Timing*
RCH Establishment and Organizational Ramp-up	Hire or assign staff necessary to support day-to-day RCH operations and coordination	**April 1, 2027-June 1,2027**
Partner Convening and Engagement	A minimum of ten (10) core partners, representing all counties across the selected ADD, formally onboarded as core partners to the RCH	

Initiative Planning and Design	RCH initiative proposals for two (2) to four (4) evidence-informed obesity and diabetes initiatives	
Initiative Implementation Planning and Budget Development	Implementation workplan, budget, and budget narrative for each approved RCH initiative	
Subrecipient Management and Pass-through Responsibilities	Procurement strategy finalized to identify key vendors and subrecipients to execute evidence-informed obesity and diabetes initiatives	
	Distribute initiative funding to subrecipients	**June 1, 2027 – September 30, 2027**
Reporting, Performance Management, and Oversight	Narrative reports of program activities, accomplishments, and financial reports	**Monthly by the 7th of each month for the previous month's activities** *First report (Contract signing through March 31) due April 7, 2027*
	Consolidated report on programmatic and financial activity for the Rural Community Hub Lead Organization and subrecipients	**Quarterly** *First report (January 31-April 30, 2027) due May 7, 2027*

***Note:** anticipated timing is provided for planning purposes; exact dates are subject to change*

IV. Subaward Relationship and Technical Assistance

Successful applicants will enter into an agreement with FHKY as a subrecipient, as defined under 2 CFR 200.331, of the CMS RHTP (Assistance Listing No. 93.978). Awards issued under this RFA will be funded through pass-through funds from a cooperative agreement awarded by CMS.

To support implementation of the RCH initiative, FHKY and CHFS will provide technical assistance and operational support to awarded Rural Community Hub Lead Organizations throughout BP1. Such support may include:

- Operational and coordination support from dedicated FHKY staff
- Technology, budget, reporting, and compliance support to assist with implementation of federal grant requirements; and

Technical assistance is intended to support and strengthen subrecipient capacity but does not replace the Rural Community Hub Lead Organization's responsibility for meeting all programmatic, financial, and compliance obligations under the subaward.

V. Anticipated Schedule and Funding

Anticipated Schedule:

The dates and milestones described in this RFA are anticipated and provided for planning purposes only. The schedule is subject to change based on program needs, administrative considerations, and required approvals.

Issuance of this RFA does not oblige FHKY to make an award by a specific date, nor does it guarantee the timing or duration of any project period.

Date	Milestone
October 13, 2026	RFA released on https://healthy-ky.org/rch
October 23, 2026	Informational webinar
October 28, 2026	Questions about RFA due to rchgrant@healthy-ky.org
November 6, 2026	Letter of Intent to Apply due to rchgrant@healthy-ky.org
November 16, 2026	Online application portal released on https://healthy-ky.org/rch
December 1, 2026	Frequently asked questions (FAQs) published
December 18, 2026	Online applications are due at 6:00 PM Eastern Daylight Time (EDT)
January 29, 2027	Anticipated notification of awards
March 1, 2027	Anticipated start date for BP1 – Rural Community Hub Launch
March 1, 2027 – September 30, 2027	BP1 – Rural Community Hub Launch
October 1, 2027 – September 30, 2031	Subsequent phases (if funded)

Anticipated Funding, Allowable Uses, and Continuation

All funding amounts described in this RFA are estimates only and are provided for planning purposes. Final award amounts, allowable uses of funds, and disbursement schedules will be determined by FHKY in accordance with CMS Rural Health Transformation Program Notice of Funding Opportunity (NOFO); the Uniform Administrative Requirements, Cost principles, and Audit Requirements under 2 CFR Part 200; the approved CMS budget, and the terms and conditions of the subaward agreement. Please see the Program-Specific Limitations Section of the NOFO for a list of allowable and unallowable costs.

For BP1 – Rural Community Hub Launch (March 1, 2027– September 30, 2027), anticipated funding may include:

- Up to $500,000 per ADD to support Rural Community Hub stand-up and operations (“Hub Operations funding”)

FHKY reserves the right to award less than the maximum amounts described above or to make no awards for a given ADD. Funds may be used only for costs that are allowable, reasonable, and allocable under the CMS NOFO and 2 CFR Part 200, are included in an approved budget, and directly support the required activities and deliverables described in this RFA. FHKY does not have authority to approve costs that are unallowable under federal requirements.

Process for distribution of initiative funding

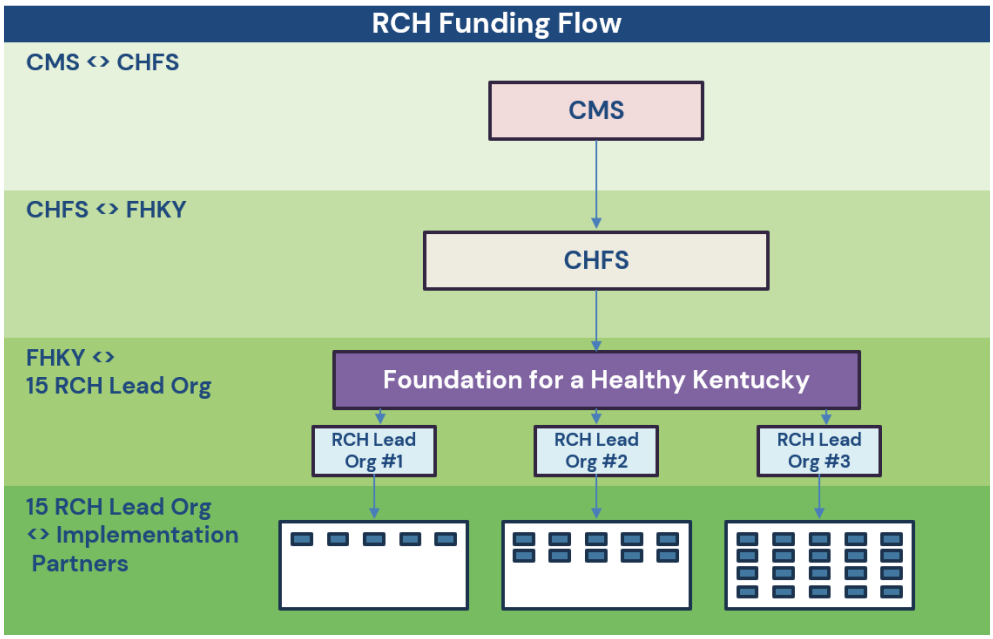


Figure 1: *RCH Funding Flow Diagram*

Subject to Kentucky's successful award and continuation through all five federal budget periods, each Rural Community Hub may be eligible to receive up to $10 million in total funding across Federal Fiscal Years 2026-2030 *(refer to **V. Anticipated Schedule and Funding Performance-based continuation** below)*.

Performance-based continuation:

The continuation of federal funding is based on performance. If the State does not meet its intended metrics for BP1, it may not receive additional funding for BP2 or it may receive less. Therefore, Kentucky's RCH initiative awards and funding for subsequent award periods are contingent upon the availability of funding at the State level, Kentucky RCH initiative annual performance, and RCH grant and contract compliance. The contracted project period 1 is expected to begin March 1, 2027, and last until September 30, 2027. Project periods are estimated.

VI. Eligibility

Eligible Applicants

Eligible applicants must be non-profit, public-benefit, or for-profit entities with demonstrated organizational capacity to serve as an ADD-level convener.

Eligible applicant types include:

- Non-profit and for-profit organizations
- Public or quasi-public entities, including local or regional governmental entities, and;
- Academic or research institutions

Applicant organizations must reside within the Area Development District (ADD) for which they are applying and maintain ongoing operations within that ADD. Organizations that reside within or maintain operations in more than one ADD may submit only one application and may apply to serve one ADD only. Applications proposing to serve more than

one ADD, or multiple applications submitted by the same organization for different ADDs, will be deemed non-responsive and will not be reviewed.

An organization's Unique Entity Identifier (UEI), tax identification number, or corporate address is not required to be located within Kentucky. Organizations whose UEI, tax identification number, or corporate address is located outside the Commonwealth remain eligible to apply, provided they meet all eligibility requirements described in this RFA and maintain ongoing operations within the ADD for which they are applying.

Applying for or being selected as a Rural Community Hub Lead Organization does not preclude an organization from applying for or participating in other Rural Health Transformation Plan (RHTP) initiative opportunities, subject to the requirements of those opportunities.

Organizational Eligibility Requirements

To be eligible for consideration, applicant organizations must also meet all of the following requirements:

- Possess a Unique Entity Identifier (UEI) issued through SAM.gov.
- Be in good standing with the Kentucky Secretary of State.
- Maintain a financial management system that complies with 2 CFR 200.302 and internal controls consistent with 2 CFR 200.303.
- Not be included on any federal exclusions lists (GSA FSD Public Knowledge Base (Tier 0) - Exclusions: How do I search for active exclusions?).
- Demonstrate a minimum of three (3) years of operating experience with physical presence in the selected ADD.

VII. How to Apply

Submission Instructions

Applications must be submitted electronically through the online application portal on https://healthy-ky.org/rch, no later than Friday, December 18, 2026 at 6:00 PM EDT. The online application portal on https://healthy-ky.org/rch will open no later than Monday, November 16, 2026.

All application components – including narrative responses and required uploads – must be completed and submitted through the online survey. It is the applicant's responsibility to ensure that the application is complete and meets all requirements. The survey questions are provided in ***Appendix A: Application Narrative*** as a reference for planning purposes only.

Application Components

The application consists of **seven (7) sections**, with a **total of 51 unique questions**, inclusive of **four (4) uploaded attachments**. **Questions 15-38** are conditional and will display in the online application portal based on the applicant's response to **Question 14** (Area Development District selection). Applicants will only be required to complete the two county-related questions associated with the selected Area Development District. The table below summarizes the number of questions and attachments in each section as presented in ***Appendix A: Application Narrative***.

Application Section	Appendix A: Application Narrative Question #s (# of unique questions)	Required Attachments
Section I: *Documentation of organizational profile and populations served*	**1-44** (22)	None
Section II: *Documentation of workforce, operational capacity, and community context*	**45-48** (4)	None
Section III: *Documentation of community partnerships and collaboration*	**49-57** (9)	At least two (2) Letters of Commitment
Section IV: *Documentation of grant management and financial capacity*	**58-65** (8)	Most recent 990 or financial statement
Section V: *Documentation of readiness to lead a Rural Community Hub*	**66-70** (5)	BP1 Workplan
Section VI: *Documentation of budget and budget narrative*	**71** (1)	BP1 Budget and Budget Narrative
Section VII: *Draft contract review and exceptions*	**72** (1)	None

For specific instructions on each question or attachment, please refer to ***Appendix A: Application Narrative***.

Guidelines for Submitted Attachments

Attachment	Guidelines
Two (2) Letters of Commitment *(Section III, Question 52)*	**Format:** Submit multiple letters as one consolidated PDF file.
Most recent 990 or financial statement *(Section IV, Question 65)*	**Format:** Submit requested documents as one consolidated PDF file.
BP1 Workplan *(Section V, Question 70)*	Applicants must upload a BP1 Workplan describing their proposed approach to meeting the Scope of Work during BP1 (Rural Community Hub Launch; March 1, 2027 – September 30, 2027). The workplan should demonstrate readiness to lead a Rural Community Hub and align with the BP1 objectives, activities, and deliverables described in ***Section III. Scope of Work***. The workplan does not need to represent a final operational plan. **Format:** PDF, suggested 5-page maximum, 1" margins, size 11 Calibri font, single-spaced

Attachment	Guidelines
BP1 Budget and Budget Narrative *(Section VI, Question 71)*	Applicants must submit a BP1 Budget and Budget Narrative covering the period of March 1, 2027 – September 30, 2027. The proposed budget and budget narrative must: • Align with the proposed BP1 Workplan and Scope of Work; • Reflect costs that are allowable, reasonable, and allocable, and; • Comply with applicable CMS budget guidance, SF-424A (Budget Information for Non-Construction Programs), and the Uniform Administrative Requirements, Cost Principles, and Audit Requirements under 2 CFR Part 200. • Include budget justification narrative **Format:** The budget must be prepared using the required budget template provided in ***Appendix B: Budget Template***. This Excel template must be converted and submitted as one consolidated PDF file prior to submission.

VIII. Application Scoring

Please note that the application includes several required questions that are not scored as standalone items. These questions are used to verify eligibility and feasibility, and to provide context for the scored sections (e.g., confirming service area and physical presence, validating financial readiness for reimbursement, and substantiating partnership commitments).

Applications will be evaluated and scored using the criteria outlined in the Response Scoring Table below. Reviewers may assign any whole-number score within the point range for each content area based on how well the application addresses the stated elements. Application scores will be used to inform funding recommendations and support a consistent and transparent review process. While application scores are a key factor in selection decisions, FHKY reserves the right to make final determinations.

Response Scoring Table

Application Section	Maximum Score
Section 1: Documentation of organizational profile and populations served *Does the applicant provide a clear, locally grounded profile of the organization, the populations served, and the organization's footprint in the selected ADD?*	**10**
Section 2: Documentation of workforce, operational capacity, and community context *Does the applicant demonstrate the workforce, operational infrastructure, programmatic experience, and data / technology readiness needed to support RCH activities?*	**20**
Section 3: Documentation of community partnerships and collaboration *Does the applicant demonstrate the organization's ability to convene core partners and network participant organizations and implement a collaborative hub-and-spoke model (collective impact potential)?*	**30**
Section 4: Documentation of grant management and financial capacity	**10**

Application Section	Maximum Score
Does the applicant demonstrate the organization's financial systems, grant management structure, and compliance practices necessary to serve as a pass-through entity under Uniform Guidance (2 CFR 200)?	
Section 5: Documentation of readiness to lead a Rural Community Hub *Does the applicant demonstrate readiness to lead a Rural Community Hub through its interpretation of Hub Lead responsibilities, ability to anticipate roadblocks, sustainability preparedness, and proposed BP1 workplan?*	**25**
Section 6: Documentation of budget and budget narrative *Does the proposed budget demonstrate a clear, compliant, and feasible use of funds for the period (March 1, 2027 – September 30, 2027)?*	**5**
Total	**100**

IX. Submission Reminders

Applications must be submitted to the online application portal on Healthy-KY.org/rch no later than Friday, December 18, 2026, at 6:00 PM EDT. Applications received after the deadline will not be considered.

X. Appendices

All appendices can be found on FHKY's website at Healthy-KY.org/rch.

- **Appendix A: Application Narrative**
- **Appendix B: Budget Template**

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